

The Blue Ocean Plan

Making the competition irrelevant

A brand-revamp strategy for Dr Bola Akinola. Not a plan to out-spend the market leader on the things he already owns, but to change the game so that the terms he competes on stop being the terms that matter. Value innovation, unmet patient needs, and the large latent demand no competitor is serving.

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The idea in one line

Do not try to out-Oni Oni. Change the game so the terms he competes on stop being the terms that matter.

The premium Lagos arthroplasty market is a small, crowded pool where everyone competes on the same few things: Western pedigree, a big procedure number, speed of recovery, robots, and a nice building. Dr Julius Oni currently wins that contest, and fighting him on it means out-spending him on what he already owns. That is a losing game.

Blue Ocean Strategy says stop fighting for a share of existing demand and create new demand in uncontested space, through value innovation: raising the value patients actually receive while lowering the cost to deliver it, at the same time. Do that well and the competition becomes irrelevant, because you are no longer selling the same thing. Oni appears here as the clearest example of the old logic, not as the finish line.

1. The red ocean: what everyone is fighting over

On a Strategy Canvas, every serious player traces almost the same curve, which is the definition of a red ocean.

Factor of competition	Industry norm	Oni's position
Western pedigree	High, rising	Very high (Johns Hopkins)
Procedure-volume claim	High	Very high (4,000+, self-reported)
Speed of recovery	High	Very high (24-hour promise)
Technology / robotics	Rising	High
Facility luxury	High	High
Price (headline)	High, opaque	High, opaque
Breadth of services	Broad	Broad (lists complex & revision)

Two things matter. First, Oni already lists complex and revision surgery, so that category is not literally empty; Dr Bola wins it only by proving depth Oni asserts on paper. Second, and more important, notice what is absent from the entire canvas: **transparency, honesty, communication, recovery support, and access**. Nobody competes on these. That absence is the opportunity.



2. What patients value is not what the industry sells

The evidence on how patients choose a surgeon points at what nobody sells:

- **Manner and communication is the single highest-rated factor (4.7/5)**, just above demonstrated quality (4.6). Cost and convenience rate lowest (3.5). Patients choose the surgeon who listens and is trusted, not the fanciest building.
- **Patients want transparency and cannot get it.** Around 96 percent believe surgeons' outcomes should be public, 95 percent their experience, 92 percent their case volume, and 80 to 95 percent want price transparency, yet only about 46 percent can find any data to compare surgeons. The largest unmet need in the market.
- **Fear drives hesitation:** fear of a poor outcome or revision, and fear of recovering alone. Remove those and you convert people currently doing nothing.
- **Word of mouth and the surgeon's personal brand dominate** the decision, far above hospital reputation and advertising. This validates leading with Dr Bola the person, and a referral engine over an ad budget.

The market sells prestige, speed and robots. Patients want to be heard, told the truth, shown the evidence, supported through recovery, and able to afford it. The gap between those two lists is the blue ocean.

3. The ERRC grid: how Dr Bola breaks away

What to eliminate, reduce, raise and create to build a new value curve without simply spending more.

ELIMINATE

- The pedigree arms race as the lead message. Let training be a quiet credential.
- Opaque, call-for-pricing cost.
- Hype and superlatives every rival already uses.

RAISE

- Radical transparency: published outcomes, case volumes, revision rates.
- Communication and dignity. The highest-rated value driver.
- Demonstrated depth in the hard cases, evidenced not asserted.

REDUCE

- Spend on facility luxury and robotics-as-marketing, which patients rate low.
- Reliance on paid advertising, which patients trust least.
- The discharge-at-the-theatre-door model.

CREATE

- Fixed, all-in, transparent package pricing. Nobody in Lagos offers it.
- A supported-recovery guarantee that removes the fear that stops people.
- Access and financing, and a before-you-fly second opinion.

Every raise and create item is wanted by patients and offered by no competitor, and several eliminate and reduce items lower Dr Bola's cost base at the same time. That is value innovation, not a spending war.



4. The new value curve

Dr Bola's canvas should look visibly different from everyone else's, not higher on the same shape. He dips low where the industry over-invests (pedigree-as-headline, facility luxury, robotics-as-marketing, advertising) and rises steeply where the industry is silent (transparency, communication, recovery support, demonstrated complex-case depth, fixed pricing, financing). A curve that diverges like that is the visual signature of a blue ocean, and it tells one coherent story: the honest, transparent, fully-supported way to have your joint replaced, at home, by the surgeon others trust with the hard cases.

5. The Buyer Utility Map: where the journey is blocked

Journey stage	The block no one removes	The blue-ocean move
Awareness	Is my pain even treatable here?	Education; honest 'is it time?' screening
Decision	No way to compare or price; fear	Radical transparency: outcomes & fixed price
Choosing	Distrust of local capability	Demonstrated depth + before-you-fly opinion
Surgery	Fear of a bad outcome or infection	Evidenced track record; revision reassurance
Recovery	I will manage this alone	Supported-recovery programme with a navigator
Aftercare	Discharged and forgotten	Structured long-term follow-up as standard
Affordability	Cost is a wall; no financing	Instalment financing; diaspora payment

The two heaviest blocks sit at the moment of decision: **opacity** (you cannot compare or price anything) and **fear** (of the outcome and of recovering alone). A strategy that removes those two does more for the practice than any amount of pedigree.



6. The real prize: the noncustomers

Stop fighting for Oni's existing patients, a small contested pool of already-converted self-payers, and unlock the far larger population not getting joint replacement at all. Three tiers, none of them targeted by any competitor.

Tier	Who they are	How to unlock them
1. Soon-to-be	Arthritis sufferers managing on painkillers, not yet considering surgery. Huge in number.	Education; an honest, accessible entry point; removal of fear
2. Refusing	Considered local surgery and said no from distrust, fear or cost. Fly abroad or suffer on.	Transparency; recovery guarantee; financing; before-you-fly opinion
3. Unexplored	Never considered that a worn joint could be fixed, especially outside the affluent core.	Community, GP, physio, employer and faith-network outreach

This is where the growth is, and it is uncontested.

7. The value-innovation moves, ranked

Move 1 (first): Radical transparency.

Publish real outcomes, actual case volumes (revision and infection specifically), complication rates, and a clear fixed price. Low cost, wanted by ~96 percent of patients, offered by no one. Against genuine data, a rival's self-reported '4,000 procedures' looks like marketing, not evidence. Highest impact, high feasibility.

Move 2: The supported-recovery guarantee.

Bundle structured aftercare, rehabilitation, a recovery navigator, and revision-risk reassurance into every replacement. Removes the biggest fear that stops people, and drives the word of mouth that actually grows the practice. High impact, high feasibility.

Move 3: Fixed-price bundles plus financing.

One transparent all-in price per joint, paired with instalment financing (on the model Duchess runs with CarePay) and a clean diaspora payment route. Instalments raise elective volume 20 to 25 percent elsewhere, and fixed bundles can be more profitable per joint when episode costs are understood. High impact, medium feasibility.

Move 4: Before-you-fly second opinion, and the salvage safety net.

An easy, low-cost second opinion for anyone weighing treatment abroad, plus being the place botched-abroad cases come home to. Catches Tier 2 at the decision and turns the outbound leak into inbound demand. Medium impact, high feasibility.

Sequence: Moves 1 and 2 first (cheap, fast, differentiating), then Move 3 (build the finance partnership), with Move 4 running alongside from the start.



8. It has been done: three proofs

- **Shouldice Hospital (hernia)**. By doing one operation only, it reached 0.8 percent recurrence versus roughly 10 percent elsewhere, 300,000 patients at 99 percent success, at about half the cost, in a warm un-clinical setting. Focus raises outcomes and lowers price at once. Lesson: narrow, deep focus on the hip and knee is a strength.
- **Narayana Health (India)**, founded by a UK-trained surgeon, delivers open-heart surgery at under \$2,000 versus \$100,000-plus in the US at comparable outcomes, via high volume, task-shifting and cross-subsidy. Lesson: focus plus volume plus cross-subsidy expands access without lowering quality.
- **Aravind Eye Care (India)** actively creates demand through community camps and outreach rather than waiting for patients, and cross-subsidises the poor at identical surgical quality. Lesson: the Tier 3 play, going to find demand, is proven at scale.

The through-line: none won by out-luxurising a rival. They won by focusing, being transparent about outcomes, expanding access, and creating demand. Exactly this plan.

9. The brand-revamp action plan

Positioning. Dr Bola (primary brand) as the honest, transparent, fully-supported way to have your joint replaced at home, by the surgeon others trust with the hardest cases. Osteon (secondary) as the place that delivers it. Complex-case authority is the credibility anchor; radical transparency is the proof; honest counsel is the voice; supported recovery and access are the felt difference.

Offer and packaging

- A single, fixed, all-in, published package price per joint replacement.
- The supported-recovery programme included as standard in every package.
- A named second-opinion service, including before-you-fly.
- A defined complex-and-revision pathway for referred and salvage cases.

Financing and access

- A lender partnership for instalment financing (study Duchess CarePay: ~40 percent deposit, 3 to 12 month terms, third-party lender).
- A clean diaspora payment route for children-abroad paying for parents.
- Later, a modest cross-subsidy so full-pay volume funds access for those who cannot pay (the Aravind and Narayana model), also a strong brand and PR asset.

Proof, digital and demand creation

- Publish the outcomes and volumes; this is the centre of gravity of the digital presence.
- A referral and testimonial engine (see the referrer-engine playbook), because word of mouth is what patients act on.
- Patient-education content serving the Tier 1 and Tier 3 noncustomers and winning search on the terms Oni is weak on (revision, failed, infected, 'do I need a replacement').
- Screening and education through GPs, physios, employers and faith and community networks.



Sequencing. Now: radical transparency and the recovery guarantee. Next: the finance partnership, diaspora payment, second-opinion service, referral and content engine. Then: community demand-creation, and consider the cross-subsidy access programme.



10. What must be validated before we commit

The strategy is sound in shape, but two honest gaps need closing with local reality before large spend:

- **No Nigeria-specific patient-value data exists.** Every study behind Section 2 is US or Indian. Validate the transparency and recovery-support drivers with a small round of Lagos patient and referrer interviews before betting the brand on them.
- **Dr Bola's own numbers must support the transparency move.** Radical transparency only works if the outcomes and volumes are genuinely strong. Confirm his real revision and infection volumes and results before publishing anything. Transparency is a weapon only when the truth is good, and for a surgeon of his training it should be.

Everything else, the fixed pricing, the recovery guarantee, the financing partnership and the second-opinion service, is buildable now and differentiating from day one.

Evidence note. Patient-value findings are peer-reviewed US and Indian studies applied to Lagos by inference; competitor claims are Oni's own marketing, described as such; frameworks are the Kim and Mauborgne canon. Some transparency figures are from shoulder rather than hip and knee arthroplasty. Directional, and to be locally validated.

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